PEER REVIEW HISTORY

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ARTICLE DETAILS

TITLE (PROVISIONAL)	Support Needs of Carers Making Proxy Healthcare Decisions for People with dementia; a systematic review based on the Noblit and Hare meta-ethnographic synthesis of qualitative studies.
AUTHORS	Hodges, Victoria; Hodges, Victoria; Lassa, Samuel; Hynes, Catherine; Mitchell, Caroline

VERSION 1 – REVIEW

REVIEWER	Stirling, Christine University of Tasmania
REVIEW RETURNED	13-May-2021

GENERAL COMMENTS	This is a useful study which should help services to focus on assisting caregivers of people with dementia when they need to make decisions. Currently the paper has gaps in the information provided about the methods and the initial search outcomes which would prevent replication of the study and omits relevant context information to guide the reader. I recommend: 1. Provide the review protocol questions in the paper even though they are in the protocol. 2. Provide more information about excluded papers - it is normal to provide numbers against categories such as - not the right patient/population 3. Review the analysis section to provide more clarity about the process - this section could benefit from referencing and examples to show how the coding through the 3 order constructs worked. 4. Provide more summary details about the included papers. This information will help the reader to navigate the results. I would expect to have seen a summary of the range of decisions the paper covered, the settings of the decisions (acute, community, primary health), the range of capacities of the people with dementia. 5. I also feel a slightly more academically critical lens was needed regarding the evidence for shared decision and stronger selection of articles for referencing.

	There were some small issues: Page 8 Line 48: was confusing – rewrite. Page 9 Line 25: approach to numerals needs to be more consistent.

REVIEWER	Légaré, France CHU de Quebec and Universite Laval
REVIEW RETURNED	20-May-2021

GENERAL COMMENTS	This review is informed by the PRISMA reporting guidelines. please use PRISMA to write your abstract add eligibility criteria in the abstract ;* Put the Background subsection before objective * Briefly present the eligibility criteria in the Methods subsection of the abstract * Specify the date of the documentary research * No “study selection” subsection * Delete the conclusion subsection * Add a discussion subsection Objective is not clear, would need to be rephrased The objective does not clearly mention the topic of dementia, unlike the objective stated in the abstract section. In addition, the authors generalize the scope of the review to care decisions other than those relating to end-of-life care "This systematic review and meta-ethnography of qualitative studies aimed to review the existing literature and develop new understanding of the support needs of carers facing health care decisions other than those relating to end of life care and to inform practitioners who may be in a position to provide support. " Methods 7-9 * Structure the "Method" section according to the PRISMA guideline * Clearly specify the eligibility criteria for the studies included in the review * The research strategy is to focus on dementia or loss of cognitive ability, which does not seem to correspond exactly to the objective as formulated in the "Introduction" section. * I suggest clarifying the elements of the PICO and reformulating the objective precisely according to the PICO * Describe the study selection process before data extraction Results 9-15 * Update the PRISMA flow chart according to the updated version PRISMA 2020 http://www.prisma-statement.org/PRISMAStatement/FlowDiagram * Rename the sub-sections of the result part according to the PRISMA2020 guideline * In view of the results presented, this is a systematic review presenting a synthesis of qualitative data based on the 7 steps of Noblit and Hare for performing a meta-ethnography. We therefore suggest simplifying the title of the article. Discussion 15-19 * Reorganize the discussion section according to PRISMA 2020

	* Rename the last paragraph "Conclusion" instead of "In conclusion" Appendices * Structure the flow chart according to the PRISMA template http://www.prisma-statement.org/PRISMAStatement/FlowDiagram * The conclusion as formulated does not mention the main needs identified by the study In addition, the authors tackled an important area also known as a decisional needs assessment. It may be useful to review the website of the OTtawa decision aids research group in Canada to have a lok at decisional needs assessment; also the prospero york university is a good place to look at current decisional needs assessment.

VERSION 1 – AUTHOR RESPONSE

Comments from reviewer 1 and 2	
Comments from reviewer 1	
Please use PRISMA to write your abstract add eligibility criteria in the abstract ;* Put the Background subsection before objective	Thank you for this suggestion and we have now edited to use PRISMA and to put the background sub-section before objective
Briefly present the eligibility criteria in the Methods subsection of the abstract	Thank you and this section has now been edited and updated under the sub-heading 'Eligibility criteria' on page 1.
Specify the date of the documentary research	We have now edited this and the date updated: Searches were repeated on the 22nd June 2021.' This updated search identified 2 extra papers included now in the table and analysis of this additional data reinforced our findings and did not change the key findings of the review.
No "study selection" subsection	This has now been included as a sub-section 'Study selection on page 6.
Delete the conclusion subsection	Sub-section deleted from page 23.
Add a discussion subsection	We have now added a sub-section 'Discussion' added on page 20
Objective is not clear, would need to be	The study aim has been added to the section Search strategy and selection strategy on page

rephrased The objective does not clearly mention the topic of dementia, unlike the objective stated in the abstract section. In addition, the authors generalize the scope of the review to care decisions other than those relating to end-of-life care "This systematic review and meta-ethnography of qualitative studies aimed to review the existing literature and develop new understanding of the support needs of carers facing health care decisions other than those relating to end of life care and to inform practitioners who may be in a position to provide support. "	5*'The objective of this study was to review the existing literature on the support needs of carers acting as proxy healthcare decision makers for people with dementia (PwD).'* In our exclusion criteria we have made it clear that this systematic review excludes papers where the sole purpose of the study was End of Life Care on page 5.
Methods 7-9 * Structure the "Method" section according to the PRISMA guideline	Thank you for highlighting this issue. We have added an additional sentence to the strengths and limitations on page 14 of our research and hope this is helpful in clarifying our approach and structure of results section: We have been guided by the report from the eMERGe reporting guidance which says there are *' no bespoke reporting guidance exists for meta-ethnography...Existing generic guidance for reporting qualitative evidence synthesis pays insufficient attention to reporting the complex synthesis processes of meta-ethnography.'* With this in mind we have not used the PRISMA in structuring the headings of the paper but have used the PRISMA checklist for both abstract and article paper in ensuring there was good practice and rigorous, comprehensive analysis, interpretation, and synthesis of the evidence*.'* (reference: France, E.F., Cunningham, M., Ring, N. et al. Improving reporting of meta-ethnography: the eMERGe reporting guidance. BMC Med Res Methodol 19, 25 (2019). https://doi.org/10.1186/s12874-018-0600-0)..
Clearly specify the eligibility criteria for the studies included in the review	This has now been specified under the sub-heading 'Eligibility criteria' on page 5. Eligibility criteria Inclusion criteria: a) People of any age and gender who have a close relative with dementia (regardless of where they reside), b) Proxy-decision making about a healthcare treatment c) Any primary, empirical research Qualitative, mixed methods Exclusion criteria: a) Published prior to 1995 b)

	Non-English language c) Only report proxy decisions not related to healthcare d) End of Life e) Reviews, editorials, dissertations, meeting abstracts and correspondence
The research strategy is to focus on dementia or loss of cognitive ability, which does not seem to correspond exactly to the objective as formulated in the "Introduction" section.	Thank you for this comment. This has been edited to reflect the focus on dementia in the rationale and research objectives on page 4. The term 'loss of cognitive ability has now been deleted.
I suggest clarifying the elements of the PICO and reformulating the objective precisely according to the PICO	Thank you and we have now created a PICO table sowing the objectives. Table 1.
Describe the study selection process before data extraction	Edited and a new section 'search strategy and selection criteria' on page 5.
Results 9-15 * Update the PRISMA flow chart according to the updated version PRISMA 2020 http://www.prisma-statement.org/PRISMAStatement/FlowDiagram	Updated and added to the supplementary PRISMA file.
Rename the sub-sections of the result part according to the PRISMA2020 guideline	Thank you for highlighting this issue. We have added an additional sentence to the strengths and limitations on page 14 of our research and hope this is helpful in clarifying our approach and structure of results section: Please see above re addition to the strengths and limitations of the research. We have been guided by the report from the eMERGe reporting guidance which says there are *' no bespoke reporting guidance exists for meta-ethnography...Existing generic guidance for reporting qualitative evidence synthesis pays insufficient attention to reporting the complex synthesis processes of meta-ethnography'* (reference: France, E.F., Cunningham, M., Ring, N. et al. Improving reporting of meta-ethnography: the eMERGe reporting guidance. BMC Med Res Methodol 19, 25 (2019). https://doi.org/10.1186/s12874-018-0600-0). With this in mind we have not used the PRISMA in structuring the headings of the results section paper but have used the PRISMA checklist for both abstract and article paper in ensuring there was good practice and rigorous, comprehensive analysis, interpretation, and synthesis of the evidence.
In view of the results presented, this is a systematic review presenting a synthesis of	We are grateful for this comment as we have now edited the title, which gives it more focus

qualitative data based on the 7 steps of Noblit and Hare for performing a meta-ethnography. We therefore suggest simplifying the title of the article.	and clarity. Title now edited 'Support Needs of Carers Making Proxy Healthcare Decisions for People with dementia; a systematic review based on the Noblit and Hare meta-ethnographic synthesis of qualitative studies.'
Discussion 15-19 * Reorganize the discussion section according to PRISMA 2020	Thank you for highlighting this issue. We have added an additional sentence to the strengths and limitations on page 14 of our research and hope this is helpful in clarifying our approach and structure of results section: We have been guided by the report from the eMERGe reporting guidance which says there are ' no bespoke reporting guidance exists for meta-ethnography...Existing generic guidance for reporting qualitative evidence synthesis pays insufficient attention to reporting the complex synthesis processes of meta-ethnography' (reference: France, E.F., Cunningham, M., Ring, N. et al. Improving reporting of meta-ethnography: the eMERGe reporting guidance. BMC Med Res Methodol 19, 25 (2019). https://doi.org/10.1186/s12874-018-0600-0). With this in mind we have not used the PRISMA in structuring the headings of the paper but have used the PRISMA checklist for both abstract and article paper in ensuring there was good practice and rigorous, comprehensive analysis, interpretation, and synthesis of the evidence. We followed the journal format in author guidelines for the discussion section.
Rename the last paragraph "Conclusion" instead of "In conclusion"	Thanks for the suggestion, however, the conclusion section has been deleted now and added to discussion section as Implications for future research and clinical practice section on page 16, as this was suggested by reviewer 1
Appendices * Structure the flow chart according to the PRISMA template http://www.prisma-statement.org/PRISMAStatement/FlowDiagram	New template now added
The conclusion as formulated does not mention the main needs identified by the study	The main needs have now been incorporated into the discussion section and implications for future research and clinical practice. Conclusion section deleted now and added to discussion section and Implications for future research and clinical practice section, page 16
Reviewer 2	

Provide the review protocol questions in the paper even though they are in the protocol.	Thank you and we have addressed this comment by adding a sub-section on research questions in the introduction section, page 4
Provide more information about excluded papers - it is normal to provide numbers against categories such as - not the right patient/population	Thank you and the details of this are now in the PRISMA diagram
3. a) Review the analysis section to provide more clarity about the process - this section could benefit from referencing and examples to show how the coding through the 3 order constructs worked. b) Provide more summary details about the included papers. This information will help the reader to navigate the results. I would expect to have seen a summary of the range of decisions the paper covered, the settings of the decisions (acute, community, primary health), the range of capacities of the people with dementia.	Thank you, Table 2 with an example of how the coding for the 3 order constructs was applied in this study and has now been included to page 8. Table 3 has been edited to show the range of decisions, the setting and capacity of the dementia patients.
5. I also feel a slightly more academically critical lens was needed regarding the evidence for shared decision and stronger selection of articles for referencing.	Thank you and we have taken your advice, appraised and added additional references in our discussion on page 15 and we think the argument is stronger now. Additional references: 31. Elissa M. Abrams, Marcus Shaker, John Oppenheimer, Ray S. Davis, Don A. Bukstein, Matthew Greenhawt, The Challenges and Opportunities for Shared Decision Making Highlighted by COVID-19, The Journal of Allergy and Clinical Immunology: In Practice,2020; 8(8)2474-2480 32. Légaré F, Canada Research Chair in Implementation of Shared Decision Making in

	Primary Care. Interprofessional approaches to shared decision making in primary care: Advancing theories, frameworks, methods and measurements. 33. Murray E, Charles C, Gafni A. Shared decision-making in primary care: tailoring the Charles et al. model to fit the context of general practice. Patient Educ Couns. 2006 Aug;62(2):205-11. doi: 10.1016/j.pec.2005.07.003. Epub 2005 Aug 31. PMID: 16139467.